National Cancer Control Programme (NCCP) 00789 · clinician draft

TCHP with Phesgo

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin combination

Active medicines: Docetaxel + Carboplatin + Pertuzumab + Trastuzumab

Emetogenic risk: High · carboplatin AUC ≥4

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1
Ondansetron	16 mg orally	Day 1 Individual dose / QT / hepatic review
Dexamethasone	12 mg orally	Day 1 Reconcile chemotherapy premedication steroids
Aprepitant	80 mg orally daily	Days 2–3
Metoclopramide	10 mg orally up to three times daily	Subsequent days: duration and use to be confirmed Individual contraindications, renal function and duration review
Dexamethasone	8 mg orally daily	Days 2–4 Carboplatin AUC ≥4 antiemetic row; reconcile total steroid exposure

Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days, starting the day before each dose; at least 3 doses before infusion. Missed doses require consultant advice; any 20 mg intravenous (IV) replacement is a consultant decision.
- Reconcile all steroid exposure and aprepitant interactions; the antiemetic steroid reference is not an instruction to add a duplicate course.
- Consider primary granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) under the source and individual risk plan.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00789 v3 · p8 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.15 · Regimen-level drafts; prescriber and oncology-pharmacy verification required

Postoperative HER2-only phase · confirm selected regimen

Active medicines: Selected postoperative HER2 medicine; separate source regimen

Emetogenic risk: Confirm the selected postoperative regimen

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Open the separately selected postoperative regimen before issuing supportive medicines. Its emetic risk, premedication and tests apply.
- Phesgo or trastuzumab alone generally needs no routine antiemetic prophylaxis. Do not apply this statement automatically to an antibody-drug conjugate.
- Pathology, including residual invasive disease, informs the next HER2 medicine and remaining treatment course.
- Continue cardiac monitoring on the selected treatment plan; surgery does not restart the clock.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00789 v3 · p1 · NCCP antiemetics v8, Table 1 pp4–5

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